

Policy on Medical Equipment Management

«Hospital Name»

Document control

Document No.	HCO/FMS/POL/05	Version	2.0
Issue No.	01	Review due	One year from implementation
Date created	_____	Date of implementation	_____
Prepared by	Engineering In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

The organisation has a programme for medical equipment management.

1. Purpose

This policy sets out how «Hospital Name» runs its medical equipment management programme. It covers:

- Planning medical-equipment procurement
- Inventorying and identifying every device
- Implementing operational and maintenance plans
- Keeping calibration current
- Ensuring only qualified personnel operate and maintain equipment
- Monitoring adverse events and recalls
- Guiding replacement and disposal

This policy covers medical equipment management specifically. Related duties — like patient assessment, clinical care, medication management, patient rights, infection control, quality and safety monitoring, or hospital governance — are covered in the hospital's other policies, not repeated here.

Words marked like this are defaults. A blank marked _____ must be filled before issue.

2. Scope

This policy applies to «Hospital Name» staff involved in facilities, utilities, medical gases, and fire or non-fire emergencies, including:

- Engineering, biomedical, nursing and departmental leaders
- The Engineering In-Charge
- The Medical Superintendent
- Departmental leaders
- The Quality Coordinator

This policy covers medical equipment management specifically. Related duties are covered in the hospital's other policies, not repeated here, including:

- Patient assessment
- Clinical care
- Medication management
- Patient rights
- Infection control
- Quality and safety monitoring
- Hospital governance

3. Policy standards

«Hospital Name» plans medical equipment against services and the strategic plan. Every device is inventoried, classified by risk and given a unique identifier. Implemented operational and maintenance plans cover operator training, daily checks and breakdown response. Calibration is current before commissioning and after every repair. Operators and maintainers are qualified. Adverse events and recalls are monitored and acted on without delay. Disposal follows written guidance.

Staff follow written guidance and keep the records listed in the traceability table.

4. Non-negotiable rules

1. Do not put medical equipment into clinical use without a unique identifier and an entry in the medical-equipment inventory.
2. Do not use medical equipment that does not have an implemented, documented operational and maintenance plan.
3. Do not use medical equipment that measures patient parameters past its scheduled calibration due date; remove it from clinical use until calibration is complete.
4. Do not allow an operator to use a medical device they have not been trained for; do not allow maintenance by personnel who are not a biomedical or instrumentation engineer or technologist with relevant training and experience.
5. Do not continue clinical use of any medical device subject to an open manufacturer or regulatory recall or hazard notice.

6. Do not condemn or dispose of medical equipment without following the written replacement-and-disposal guidance.
7. Do not bypass the stop-work authority in section 6 when the trigger conditions are met.
8. Staff who see a FMS.5 rule broken report it the same shift to the Engineering In-Charge or the Medical Superintendent.

5. What we do

5.1 The organisation plans for medical equipment in accordance with its services and strategic plan.

The organisation plans medical equipment to match services and the strategic plan, including future needs. Indian Public Health Standards are a reference for a minimum set. Selection, rental, update or upgrade is collaborative (end-user, management, finance, engineering, biomedical).

The Engineering In-Charge holds the current medical-equipment plan with the Medical Superintendent.

5.2 Medical equipment is inventoried and proper logs are maintained as required.

Medical equipment is inventoried and logs are maintained. Items are classified by medical-device risk. Each has a unique identifier, including rental and demonstration items. Factory test and conformance certificates are kept.

An in-use device with no identity does not satisfy this element.

5.3 The documented operational and maintenance (preventive and breakdown) plan for medical equipment is implemented.

The documented operational and maintenance (preventive and breakdown) plan for medical equipment is implemented: operator training, daily operating checks, preventive-maintenance tracker, and breakdown response including nights and weekends.

Using equipment with no implemented plan is a stop-work trigger (section 6).

5.4 Medical equipment is periodically inspected and calibrated for their proper functioning.

Medical equipment that measures is inspected and calibrated on a weekly, monthly or annual schedule as the manufacturer and this hospital define, in-house or outsourced, with traceability. Conformance is checked before commissioning and again after repair.

A measuring device past its calibration due date is a stop-work trigger (section 6).

5.5 Qualified and trained personnel operate and maintain medical equipment

Qualified and trained personnel operate and maintain medical equipment. Operators are trained for the devices they use (for example blood-gas analyser, electrocardiograph, syringe pump). Maintenance is by a biomedical engineer or technologist, or an instrumentation engineer or technologist, with relevant training and experience.

5.6 Written guidance supports medical equipment replacement and disposal

Written guidance supports medical equipment replacement and disposal. Condemnation is systematic and recorded.

5.7 There is monitoring of medical equipment and medical devices related to adverse events, and compliance hazard notices on recalls.

Medical equipment and devices are monitored for adverse events. Hazard notices and recalls from the manufacturer or regulator are acted on at once; the device is not returned to clinical use until the issue is closed. The Guidebook names Gazette of India GSR 78(E) 2023 / Medical Devices Rules 2023 and participation in the Materiovigilance Programme of India. Medication-supply recall is covered in other hospital policies; this element is equipment and devices.

An open recall left in use is a stop-work trigger (section 6).

5.8 Downtime for critical equipment breakdown is monitored from reporting to inspection and implementation of corrective actions.

Downtime for critical medical-equipment breakdown is monitored from reporting to inspection and corrective action. At a minimum the critical list includes ventilators, X-ray, magnetic resonance imaging, catheterisation laboratory, computed tomography, anaesthesia machines, monitors, laboratory and ultrasound — especially where there is no alternative. A complaint register records receipt, job allotment and user-ratified completion.

Start of downtime is complaint time; end is user-ratified completion.

6. Stop-work authority

Do not use medical equipment that is past the due date for preventive maintenance or calibration, or that is under an open manufacturer or regulatory recall or hazard notice.

Stop-work applies to putting that equipment into clinical use. Immediate life-saving care uses the next safe alternative and is documented.

The person who stops tells the Engineering In-Charge and the Medical Superintendent the same shift. Refusing overdue or recalled equipment is not a disciplinary matter.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable that FMS.5 is resourced and

	followed.
Engineering In-Charge	Owns day-to-day implementation and records for this standard.
Quality Coordinator	Audits this policy quarterly; holds training acknowledgements.
departmental leaders	Run the department-level duties this standard names.

8. Quality monitoring (RCA → CAPA)

The Quality Coordinator audits this policy quarterly. The audit reviews:

- Records for a sample of this standard's objective elements, checked against the What-we-do steps.
- Documentary evidence is on file for each asterisked objective element in the sample.
- CORE objective elements show no critical gaps in the sample.
- Stop-work events (if any) are logged with outcome.

Root-cause analysis is required when a gap found in this audit remains open beyond 90 days.

This policy is reviewed annually, and sooner after a related facility change, utility failure, equipment recall or fire-plan change.

9. Training and staff acknowledgement

Staff covered by this policy are trained at induction and once a year after that. Training covers the What-we-do steps, non-negotiables and stop-work (if present).

Staff acknowledgement

I have read the Policy on Medical Equipment Management of «Hospital Name». I will follow the processes described.

Name: _____ Designation: _____

Department / floor: _____ Date: _____

Signature: _____

(One row per staff member. The Quality Coordinator holds signed acknowledgements with the induction record.)

10. Distribution

This policy is distributed to:

- Medical Superintendent
- Engineering In-Charge

- Quality Coordinator
- departmental leaders
- staff covered by FMS.5

11. Abbreviations

Abbreviation	Meaning
CAPA	Corrective and Preventive Action
CORE	Core objective element (NABH)
FMS	Facility Management and Safety (NABH Hospitals 6th Edition chapter)
HCO	Hospital (Full Accreditation programme under NABH Hospitals 6th Edition)
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OE	Objective Element

12. Traceability to NABH HCO Full Accreditation 6th Edition FMS.5

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

OE	Level	Requirement	Where addressed	Responsible
FMS.5.a	Commitment	The organisation plans for medical equipment in accordance with its services and strategic plan.	Section 3; 5.1	Engineering In-Charge
FMS.5.b	Commitment	Medical equipment is inventoried and proper logs are maintained as required.	Section 3; 5.2	Engineering In-Charge
FMS.5.c	CORE*	The documented operational and maintenance (preventive and breakdown) plan for medical equipment is implemented.	Section 3; 5.3; Section 6 Stop-work	Engineering In-Charge
FMS.5.d	Commitment	Medical equipment is periodically	Section 3; 5.4; Section 6 Stop-work	Engineering In-Charge

		inspected and calibrated for their proper functioning.		
FMS.5.e	Commitment	Qualified and trained personnel operate and maintain medical equipment.	Section 3; 5.5	Engineering In-Charge
FMS.5.f	Commitment*	Written guidance supports medical equipment replacement and disposal.	Section 3; 5.6	Engineering In-Charge
FMS.5.g	Commitment*	There is monitoring of medical equipment and medical devices related to adverse events, and compliance hazard notices on recalls.	Section 3; 5.7; Section 6 Stop-work	Engineering In-Charge
FMS.5.h	Achievement	Downtime for critical equipment breakdown is monitored from reporting to inspection and implementation of corrective actions.	Section 3; 5.8	Engineering In-Charge

13. Required Records / Evidence Checklist

Records the hospital holds under this policy, listed by objective element.

FMS.5.a — The organisation plans for medical equipment in accordance with its services and strategic plan.

- Written medical-equipment plan matched to services and the strategic plan, referencing the Indian Public Health Standards minimum set.
- Collaborative-selection record (end-user, management, finance, engineering, biomedical).
- Periodic review record of the plan.

FMS.5.b — Medical equipment is inventoried and proper logs are maintained as required.

- Medical-equipment inventory classified by device risk, with a unique identifier including rental and demonstration items.
- Factory test or conformance certificate on file.
- In-use device check confirming no device without an identity.

FMS.5.c — The documented operational and maintenance (preventive and breakdown) plan for medical equipment is implemented.

- Documented operational and maintenance (preventive and breakdown) plan for medical equipment.
- Operator-training record and daily operating-check log.
- Breakdown-response record, including nights and weekends.

FMS.5.d — Medical equipment is periodically inspected and calibrated for their proper functioning.

- Calibration schedule (weekly, monthly or annual as the manufacturer defines) with traceability.
- Pre-commissioning and post-repair conformance-check record.
- Calibration-due tracking log confirming no overdue device in clinical use.

FMS.5.e — Qualified and trained personnel operate and maintain medical equipment.

- Operator-training record per device type (for example blood-gas analyser, electrocardiograph, syringe pump).
- Biomedical or instrumentation engineer/technologist qualification record for maintenance staff.
- Training-currency check for operators and maintainers.

FMS.5.f — Written guidance supports medical equipment replacement and disposal.

- Written medical-equipment replacement and disposal guidance.
- Condemnation record, systematically applied.
- Disposal log.

FMS.5.g — There is monitoring of medical equipment and medical devices related to adverse events, and compliance hazard notices on recalls.

- Adverse-event and hazard-notice/recall log for medical equipment and devices.
- Record showing a recalled device was withdrawn from clinical use until the issue closed.
- Materiovigilance Programme of India participation record, where applicable.

FMS.5.h — Downtime for critical equipment breakdown is monitored from reporting to inspection and implementation of corrective actions.

- Complaint register for critical medical equipment (ventilators, X-ray, MRI, cath lab, CT, anaesthesia machines, monitors, laboratory, ultrasound).
- Downtime-duration record from reporting to corrective action.
- Alternative-equipment-use record where no backup device exists.

14. References

- National Accreditation Board for Hospitals and Healthcare Providers (NABH), Accreditation Standards for Hospitals, 6th Edition (January 2025) — Facility Management and Safety, standard FMS.5.
- NABH Guidebook to Accreditation Standards for Hospitals, 6th Edition — FMS.5 interpretations.
- Gazette of India. GSR 78(E), 2023 — Medical Devices (Amendment) Rules, 2023. Referenced for statutory adverse-event monitoring, hazard notices and recall handling for medical equipment and devices.
- Central Drugs Standard Control Organisation (CDSCO), Government of India. Materiovigilance Programme of India (MvPI). Referenced for participation in adverse-event reporting for medical devices.

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked _____ must be completed before the document is issued.

Several requirements in this document are statutory rather than advisory — in particular those arising under the Medical Devices Rules as cited in NABH FMS.5 (Gazette of India GSR 78(E) 2023) including adverse-event monitoring, hazard notices and recalls. Statutory requirements change, and State authorities impose additional or stricter conditions. «Hospital Name» is responsible for verifying the current text of any rule cited here and the conditions attached to its own authorisations and licences; this document does not constitute legal advice.

«Hospital Name» remains responsible for verifying that it is current and consistent with the edition of the accreditation standard against which it is being assessed. The clinical and technical content reflects recognised national and international guidance current at the date of preparation.

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